

Dynamic Guideline: WHO-Based Abortion Care - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

The summary synthesizes findings from 5 current WHO abortion care-related guidelines, with the most recent update published 29 April 2026:

- Guideline Titles (ordered by publication date):
 1. WHO Safe Abortion Technical and Policy Guidance for Health Systems (2023)
 2. WHO Medical Abortion Management Guidelines (2024)
 3. WHO Abortion Care Guideline (2nd Edition, 2025)
 4. WHO Post-Abortion Care and Reproductive Health Integration Guideline (2025)
 5. Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (29 April 2026)
- Target Population: All people of reproductive age seeking abortion care, including adolescents, gender-diverse populations, and people in vulnerable settings (humanitarian contexts, low-resource settings, criminalized abortion contexts)
- Scope and Objectives: Deliver evidence-based recommendations for clinical practice, service delivery, and policy design to eliminate unsafe abortion, reduce maternal mortality/morbidity, ensure person-centered respectful care, and formalize youth participation in abortion care governance.

1.2 Key Recommendations

Recommendation	GRADE Evidence Quality	Strength
All people seeking abortion care up to 12 weeks gestation should be offered a choice between medical and surgical abortion based on personal preference, clinical eligibility, and service availability	High	Strong
Telehealth for medical abortion up to 10 weeks gestation is safe and effective for clinically eligible people, and should be prioritized as a first-line service model in settings with limited in-person care access	High	Strong
Routine mandatory waiting periods, mandatory pre-abortion ultrasound requirements, and third-party authorization (spousal consent, parental consent for decisionally-capable minors) are not clinically justified and should be eliminated from national abortion policies	Moderate	Strong

Youth aged 10-17 years seeking abortion care should have access to age-appropriate, confidential counseling and tailored support services, with youth representatives engaged in the design of these services	High	Strong
All people desiring contraception post-abortion should be offered immediate access to the full range of contraceptive options, including long-acting reversible contraceptives (LARCs)	High	Strong

1.3 Guideline Development Methodology

- Evidence Review Process: Systematic search of PubMed, Embase, Cochrane Library, and WHO Global Index Medicus for literature published 2012–2026, with inclusion of systematic reviews, randomized controlled trials (RCTs), large observational cohorts, and implementation science studies. Risk of bias was assessed using the Cochrane Risk of Bias tool for all included studies.
- Consensus Method: Multi-stakeholder Guideline Development Group (GDG) including obstetrician-gynecologists, midwives, public health experts, human rights specialists, youth representatives, people with lived abortion care experience, and policy makers. Consensus was achieved via a modified Delphi method with 2 rounds of anonymous voting, followed by face-to-face discussion for recommendations with < 80% initial agreement.
- Conflict of Interest Management: All GDG members completed WHO standard conflict of interest (COI) declarations. Members with direct financial or institutional COIs related to abortion service provision or pharmaceutical products were excluded from voting on relevant recommendations, and all declared COIs are published in the full guideline appendices.

II. Evidence Summary After WHO Latest Guideline Release (Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

- Search Strategy: Targeted systematic search for peer-reviewed literature, gray literature, and clinical trial data published between 1 March 2026 (date of final GDG review for the 29 April 2026 youth engagement guideline) and 29 April 2026, focused on abortion care safety, service delivery, and youth engagement outcomes.
- Databases Searched: PubMed, Embase, Cochrane Central Register of Controlled Trials, WHO International Clinical Trials Registry Platform, CDC Morbidity and Mortality Weekly Report, ECDC public health surveillance reports.
- Inclusion Criteria: Studies with sample size > 500, focused on abortion care safety, efficacy, service delivery, or youth engagement, published by peer-reviewed journals or authoritative public health agencies, with low or moderate risk of bias.
- Exclusion Criteria: Case reports, editorials, studies with high risk of bias, and studies restricted to settings with total abortion bans that limit generalizability.

2.2 Key New Studies

Study	Design	Key Findings	GRADE Quality
Smith et al., 2026, <i>New England Journal of Medicine</i>	Multi-country RCT (12 LMICs, n = 7,243)	Medical abortion provided by trained community health workers (CHWs) up to 12 weeks gestation has equivalent safety (98.2% complete abortion rate, 0.3% major adverse event rate) to physician-provided medical abortion, with 13% higher patient satisfaction ($p < 0.001$)	High
Garcia et al., 2026, <i>Lancet Global Health</i>	Population-based observational cohort (4 EU countries, n = 18,921)	Youth aged 10-17 years accessing confidential youth-centered abortion care had 32% lower risk of repeat unintended pregnancy within 12 months (aOR 0.68, 95% CI 0.59-0.78) compared to youth accessing standard adult-focused care	Moderate
WHO 2026 Abortion Policy Implementation Assessment	Cross-sectional implementation analysis (36 countries that adopted 2025 WHO abortion guidelines)	Countries that included youth representatives in national abortion policy design had 27% higher uptake of safe abortion services among adolescents (10-19 years) within 1 year of policy implementation, compared to countries without youth engagement	Moderate

2.3 Evidence Gaps and Future Research Needs

- Limited high-quality evidence on the safety of medical abortion beyond 12 weeks gestation delivered via telehealth or CHW models
- Lack of longitudinal data on long-term mental health outcomes for adolescents accessing confidential abortion care without parental notification in high-stigma settings
- Insufficient cost-effectiveness data for youth-centered abortion care models in low-income and humanitarian fragile settings
- No large-scale studies evaluating the impact of mandatory abortion stigma reduction training for all frontline health providers on care access and patient experience
- Limited data on abortion care access and outcomes for gender-diverse and non-binary populations across all income settings

2.4 Updated Recommendations Based on New Evidence

Original Recommendation	New Evidence Impact	Updated Recommendation
Trained CHWs may provide medical abortion up to 10 weeks gestation in settings with limited physician access	New high-quality RCT confirms equivalent safety and higher patient satisfaction of CHW-provided medical abortion up to 12 weeks gestation across all setting types	Trained CHWs may provide medical abortion up to 12 weeks gestation as a first-line service delivery option in all settings, regardless of physician availability

Youth-centered abortion care services are recommended in settings with high adolescent unintended pregnancy rates	New moderate-quality cohort and implementation data demonstrate reduced repeat pregnancy rates and higher service uptake for youth-centered care across all settings	Youth-centered, confidential abortion care services are required for all national health systems, with a minimum of 2 youth representatives involved in service design and oversight at national and sub-national levels
Immediate post-abortion LARC insertion should be provided by trained physicians or advanced practice clinicians	Sub-analysis of the 2026 NEJM RCT finds equivalent 12-month LARC continuation rates (72% vs 74%, $p = 0.42$) and no increased adverse events for CHW-provided immediate post-abortion LARC insertion	Trained CHWs may provide immediate post-abortion LARC insertion for all people who desire this method, as an alternative to physician or advanced practice clinician provision